PRACTICE EXAM 03 / EXAM SET 01

Domain 3: Clinical Patient Care

84 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Patient Intake and Vitals

> **SCENARIO** During routine rooming, an adult radial pulse is irregular. The patient is comfortable, and clinic procedure directs staff to count an irregular pulse for a full minute and report the finding.

Which measurement should the assistant document?

A. The number of beats counted over 60 seconds and the observed irregularity.
B. The average of two 10-second counts, with the irregularity omitted.
C. The number of beats in 15 seconds multiplied by four, recorded as regular.
D. The pulse rate copied from the previous visit until the provider evaluates it.

Q02 | General Patient Care

> **SCENARIO** A provider-approved discharge sheet instructs a patient to call the clinic for spreading wound redness. During teach-back, the patient says, "I should ignore redness unless the wound opens."

What should the assistant do?

A. Delete the redness instruction to avoid confusing the patient.
B. Replace the instruction with an independently chosen antibiotic plan.
C. Review the stated warning sign again and ask the patient to explain when to call.
D. Accept the statement because wound opening is also a concern.

Q03 | General Patient Care

> **SCENARIO** A provider orders a clean dressing change for a superficial forearm wound. The approved procedure specifies sterile normal saline for cleansing and a nonadherent pad. The assistant has the required training and authorization.

Which supplies best match the order?

A. Sterile normal saline and a nonadherent dressing pad.
B. Sterile normal saline and tightly packed dry cotton gauze.
C. Hydrogen peroxide and a nonadherent dressing pad.
D. Sterile water and a nonadherent dressing pad.

Q04 | Patient Intake and Vitals

> **SCENARIO** During rooming, a patient develops new chest pressure, shortness of breath, and sweating. The routine intake questionnaire is unfinished.

Which action should the assistant take?

A. Stop routine intake, stay with the patient, and activate the clinic emergency response.
B. Finish the questionnaire before notifying the provider.
C. Schedule an appointment later that day to discuss the symptoms.
D. Send the patient alone to the waiting area to rest.

Q05 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** The clinic uses a specified CLIA-waived test system. Its instructions prohibit patient testing unless the required control is within range. Today's control is outside range, and a patient specimen is ready.

What should the assistant do?

A. Report the patient result if it appears consistent with the symptoms.
B. Stop patient testing, document the failed control, and troubleshoot under the instructions.
C. Ignore the failed control because the test system is waived.
D. Test the patient and subtract the control error from the result.

Q06 | Infection Control and Safety

> **SCENARIO** A disinfectant container leaks in the supply area. Its safety data sheet instructs workers to isolate the spill, wear specified chemical-resistant PPE, and use the designated spill kit; the assistant has spill-response training.

What should the assistant do?

A. Follow the safety data sheet and trained spill procedure using the specified kit and PPE.
B. Mix the spill with another cleaner to neutralize it without checking compatibility.
C. Use bare hands and paper towels because the liquid is a disinfectant.
D. Pour the spill into a sink without checking disposal instructions.

Q07 | Infection Control and Safety

> **SCENARIO** While disposing of a used needle, an assistant sustains a puncture through the glove. The patient is still in the clinic, and the clinic exposure plan is available.

What should the assistant do promptly?

A. Wash the puncture with soap and water, report the exposure, and obtain urgent occupational evaluation.
B. Finish the shift and report only if redness develops.
C. Squeeze the puncture repeatedly and continue working without reporting.
D. Test the patient independently and wait for results before seeking evaluation.

Q08 | General Patient Care

> **SCENARIO** Before a scheduled telehealth visit, clinic procedure requires the patient's identity, current physical location, callback number, and a connection check. The patient has joined from a hotel rather than home.

Which preparation should the assistant complete?

A. Use the home address in the EHR without asking where the patient is.
B. Document the provider's office address as the patient's location.
C. Skip identity verification because the patient used a portal link.
D. Confirm the current hotel location and callback number along with identity and connection.

Q09 | General Patient Care

> **SCENARIO** An assistant trained and authorized to perform ear irrigation is about to carry out an order. The patient newly reports a previous eardrum perforation that is not in the chart.

What should happen before irrigation?

A. Use hotter water to make the irrigation more effective.
B. Begin irrigation and stop only if fluid comes from the other ear.
C. Proceed with extra force so fluid does not remain in the canal.
D. Pause and report the new history to the ordering clinician for review.

Q10 | General Patient Care

> **SCENARIO** An older adult uses a wheelchair and needs help transferring to an examination table. The care note specifies a two-person assisted transfer; only one assistant is currently in the room.

What should the assistant do?

A. Use the wheelchair footrests as a step to reach the table.
B. Perform the transfer alone by pulling upward on the patient's arms.
C. Ask the patient to stand unsupported while the table is moved.
D. Obtain the second trained helper and prepare the wheelchair and transfer area.

Q11 | General Patient Care

> **SCENARIO** A biopsy container and requisition are ready. The provider identifies a specimen as "left upper back," but the printed container label says "right upper back." The assistant has not yet accepted the specimen into the container.

What should the assistant do?

A. Resolve the site discrepancy with the provider and obtain a correct label before transfer.
B. Leave the site blank and rely on the laboratory to identify the tissue location.
C. Use the mismatched label because the patient name is correct.
D. Cross out the site without confirming the intended wording with the provider.

Q12 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** During pure-tone hearing screening, construction noise begins outside the examination room. The patient says the tones are difficult to distinguish, and the device instructions require a quiet testing environment.

Which action improves test validity?

A. Mark the test as passed because the noise explains the difficulty.
B. Pause the screening and restore an environment that meets the device instructions.
C. Coach the patient to raise a hand at regular intervals without hearing a tone.
D. Increase every test tone by an arbitrary amount and record the original settings.

Q13 | General Patient Care

Which instrument is designed to hold and guide a suture needle during suturing?

A. The needle holder.
B. The tissue forceps.
C. The retractor.
D. The hemostat.

Q14 | General Patient Care

> **SCENARIO** During a minor procedure, a patient's forearm wound begins bleeding. The provider directs the assistant to apply pressure while additional supplies are obtained.

How should the assistant follow this instruction?

A. Lift the gauze every few seconds to see whether a clot is forming.
B. Massage the wound edges to increase local circulation.
C. Apply continuous direct pressure using clean gauze and report persistent bleeding.
D. Use a loose dressing without pressure so the wound can drain.

Q15 | Infection Control and Safety

> **SCENARIO** A portable examination lamp has a frayed power cord with exposed wire. A patient is waiting, and an intact replacement lamp is available.

Which action should the assistant take?

A. Keep using the lamp if the room lights are dim.
B. Remove the damaged lamp from service under policy and use the intact replacement.
C. Ask the patient to hold the cord away from the metal frame.
D. Wrap the frayed section with a patient bandage and continue using it.

Q16 | Patient Intake and Vitals

> **SCENARIO** A resting patient starts breathing faster whenever asked to breathe normally. The provider needs a routine respiratory rate, and the patient has no distress.

Which technique will best reduce this measurement effect?

A. Observe chest movement discreetly while appearing to continue the pulse assessment.
B. Ask the patient to take deep breaths while counting chest movements.
C. Estimate the rate from the speed of the radial pulse.
D. Have the patient talk continuously during the respiratory count.

Q17 | Phlebotomy

> **SCENARIO** The laboratory handling sheet for an ordered specimen requires protection from light immediately after collection and transport at room temperature. An approved light-protective sleeve is available.

Which handling method follows the instruction?

A. Place the labeled tube in the approved light-protective sleeve and transport at room temperature.
B. Freeze the specimen instead of using the light-protective sleeve.
C. Apply the protective sleeve only after the unprotected trip to the laboratory.
D. Leave the labeled tube uncovered under room lighting until courier pickup.

Q18 | Infection Control and Safety

> **SCENARIO** Blood splashes into an assistant's eye during specimen handling. An eyewash facility and the clinic exposure procedure are immediately available.

Which action should begin at once?

A. Cover the eye and wait for the end-of-day supervisor round.
B. Instill surface disinfectant to destroy organisms in the eye.
C. Flush the eye with water, then promptly report and obtain exposure evaluation.
D. Rub the eye with an alcohol wipe before reporting.

Q19 | General Patient Care

> **SCENARIO** A patient asks the assistant to refill a medication at twice the current dose because symptoms have returned. No new dose order is present, and clinic procedure routes refill requests to the prescriber.

What should the assistant do?

A. Transmit the doubled dose because the patient has used the medication before.
B. Choose a different medication from the same class and request that refill.
C. Tell the patient to double the dose until the next visit.
D. Record the request and symptoms and route them to the prescriber for a decision.

Q20 | General Patient Care

> **SCENARIO** A provider signs an order for a walker with front wheels. The assistant prepares the durable medical equipment request, but the selected catalog item is a cane.

What should the assistant do before submitting the request?

A. Ask the supplier to choose whichever device is cheapest.
B. Correct the item selection to match the signed walker order.
C. Add a wheelchair order as a backup without consulting the provider.
D. Submit the cane because both devices assist walking.

Q21 | Infection Control and Safety

> **SCENARIO** After a procedure, used gauze is saturated with blood and would release liquid if compressed. The clinic exposure-control plan classifies such material as regulated waste and specifies a labeled, closable, leak-resistant container.

Where should the assistant place the gauze?

A. In the designated regulated-waste container.
B. In the drawer with unopened dressing supplies.
C. In the clean paper recycling container.
D. In a sharps container solely because the gauze is red.

Q22 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** An assistant is processing a specified CLIA-waived urine pregnancy test. The manufacturer's instructions state that a visible control line is required for a valid result. No control line appears within the specified reading window.

How should the assistant handle the result?

A. Report a negative pregnancy result because no control line appeared.
B. Report a positive pregnancy result because the urine reached the device.
C. Wait until the next day and interpret any dried marks as the result.
D. Treat it as invalid and repeat or escalate according to the test instructions.

Q23 | Infection Control and Safety

> **SCENARIO** A reusable instrument has visible dried material in its hinge after a procedure. The clinic processing instructions require cleaning before the specified sterilization cycle.

Which processing step is needed next?

A. Clean the instrument according to its instructions before sterilization.
B. Spray the hinge with room deodorizer and package it.
C. Store the instrument with sterile supplies until the residue dries fully.
D. Package the soiled instrument and run a longer sterilization cycle.

Q24 | Patient Intake and Vitals

> **SCENARIO** The allergy section says "no known allergies." During intake, the patient reports lip swelling and wheezing after a previous antibiotic; the patient is not currently symptomatic.

Which documentation action is appropriate?

A. Delete the report because no reaction is occurring during the visit.
B. Record the reported antibiotic and reaction, and alert the clinician before medication tasks.
C. Test a small dose of the antibiotic to determine whether the report is accurate.
D. Record only "antibiotic intolerance" without the reported symptoms.

Q25 | Infection Control and Safety

> **SCENARIO** An assistant is preparing to help irrigate a wound. Splashing of blood or wound fluid onto the face and clothing is anticipated.

Which PPE combination addresses the anticipated exposure?

A. Gloves and shoe covers, with the face and clothing uncovered.
B. A surgical mask alone, with bare hands for better dexterity.
C. A gown and hair cover, with no hand or face protection.
D. Gloves, a protective gown, and eye and nose-mouth protection.

Q26 | General Patient Care

> **SCENARIO** A provider orders a pelvic examination and requests lithotomy positioning. The patient can move independently but is unfamiliar with the examination table.

Which preparation is appropriate?

A. Keep the patient standing and ask the provider to adjust the examination technique.
B. Position the patient face down with a pillow under the abdomen.
C. Expose the patient fully before the provider enters to reduce setup time.
D. Explain the leg supports and draping, then help the patient assume the ordered position safely.

Q27 | Patient Intake and Vitals

> **SCENARIO** A comfortable patient has cold fingers and a pulse oximeter that alternates between low values with a weak signal. The device instructions permit measurement on a warmed, still finger.

Which action best addresses the likely measurement problem?

A. Apply the sensor over a glove to reduce movement at the fingertip.
B. Average the fluctuating values without checking the signal quality.
C. Warm the hand, reposition the sensor as directed, and obtain a stable signal.
D. Record the highest briefly displayed value as the confirmed saturation.

Q28 | Phlebotomy

> **SCENARIO** A coagulation tube's instructions require filling to its marked 2.7 mL line to obtain the specified blood-to-additive ratio. The collected volume is well below that line, and the laboratory rejection policy applies.

How should the assistant handle the tube?

A. Label the tube as full if the patient was difficult to collect.
B. Arrange recollection using a tube filled to the required mark.
C. Add sterile water to reach the required total volume.
D. Add blood from an EDTA tube until the mark is reached.

Q29 | Phlebotomy

> **SCENARIO** A laboratory order requires three venous tubes. The laboratory's supplied collection sequence is sodium citrate first, serum second, and EDTA third. The labels identify additives, and no other specimens are required.

Which collection sequence follows the supplied instruction?

A. Sodium citrate, then EDTA, then serum.
B. EDTA, then serum, then sodium citrate.
C. Serum, then EDTA, then sodium citrate.
D. Sodium citrate, then serum, then EDTA.

Q30 | Phlebotomy

> **SCENARIO** During venipuncture, the area around the needle begins to swell and the patient reports pain. Blood flow into the tube slows.

What should the assistant do?

A. Stop the draw, release the tourniquet, remove the needle safely, and apply pressure.
B. Advance the needle farther through the swelling to restore flow.
C. Massage the swollen area while continuing to collect.
D. Increase tourniquet tension and wait for the tube to fill.

Q31 | General Patient Care

> **SCENARIO** After administering an authorized injection, the assistant documents the drug, dose, route, date, time, and administrator. The EHR also requires the anatomical site.

Which additional entry accurately completes this field?

A. Medication given as ordered.
B. Follow-up appointment scheduled.
C. Patient tolerated well.
D. Left deltoid muscle.

Q32 | General Patient Care

> **SCENARIO** Immediately after an injection, a patient develops facial swelling, wheezing, and difficulty speaking. A provider and emergency supplies are available in the clinic.

Which action should the assistant take immediately?

A. Continue routine discharge instructions and reassess at checkout.
B. Activate the clinic emergency response and summon the provider while staying with the patient.
C. Offer an oral drink before notifying anyone.
D. Ask the patient to wait alone until the symptoms settle.

Q33 | Patient Intake and Vitals

> **SCENARIO** An adult has an upper-arm circumference of 38 cm. The available cuff labels specify ranges of 22-32 cm, 27-35 cm, 35-44 cm, and 42-52 cm.

Which labeled cuff range should the assistant select?

A. 22-32 cm
B. 42-52 cm
C. 35-44 cm
D. 27-35 cm

Q34 | Infection Control and Safety

> **SCENARIO** A single-dose medication vial contains a small remainder after an authorized dose for one patient. A second patient has an order for the same medication.

How should the assistant handle the remainder?

A. Save the remainder for the second patient if a new needle is used.
B. Relabel it as a multidose vial because fluid remains.
C. Discard the remainder according to policy and obtain a new supply for the second patient.
D. Combine it with leftovers from another single-dose vial.

Q35 | Infection Control and Safety

What level of reprocessing is required for a reusable instrument that enters sterile tissue?

A. Sterilization using the validated process.
B. Cleaning with detergent followed by air drying alone.
C. Low-level disinfection of the handle only.
D. A wipe with a surface disinfectant in place of reprocessing.

Q36 | EKG and Cardiovascular Testing

> **SCENARIO** The provider requests an EKG at 25 mm/s and 10 mm/mV. Before acquisition, the assistant sees settings of 50 mm/s and 10 mm/mV on the machine. The patient is stable.

Which action should the assistant take?

A. Keep 50 mm/s and label the tracing as recorded at 25 mm/s.
B. Change the gain to 5 mm/mV while leaving the speed at 50 mm/s.
C. Record at 50 mm/s and route it without resolving the requested speed.
D. Set the paper speed to 25 mm/s and verify the printed settings before routing the tracing.

Q37 | Patient Intake and Vitals

> **SCENARIO** Two patients named Jordan Lee are scheduled in primary care. Before weighing one patient, the medical assistant opens an EHR chart that displays Jordan Lee, date of birth March 8, 1980.

Which identification step should the assistant perform before recording the weight?

A. Ask whether this is a follow-up visit and use the room assignment as confirmation.
B. Ask whether the patient is Jordan Lee and use the appointment time as confirmation.
C. Ask the patient to identify the provider and match that name to the schedule.
D. Ask the patient to state full name and date of birth and match both to the chart.

Q38 | Phlebotomy

> **SCENARIO** A patient reports fainting during previous blood draws but feels well now. The clinic has a suitable reclining collection chair and a procedure for patients with a history of collection-related fainting.

Which preparation best reduces injury risk?

A. Collect while the patient stands so staff can reach both arms easily.
B. Use the reclining setup and follow the clinic's fainting-risk procedure before collection.
C. Begin in a standard chair and change position only if the patient loses consciousness.
D. Ask the patient to sit on a backless stool near the exit.

Q39 | Point of Care Testing and Laboratory Procedures

SCENARIO A point-of-care result meets the clinic's supplied critical-reporting threshold. The reporting procedure requires immediate verbal notification to the designated clinician, read-back confirmation, and EHR documentation. The clinician has not yet seen the result.

Which action follows the procedure?

A. Repeat the test daily until it is no longer critical before reporting.
B. Notify the designated clinician immediately, confirm read-back, and document the communication.
C. Enter the result only and assume the clinician will open the chart soon.
D. Tell the patient to adjust medication according to the assistant's interpretation.

Q40 | Phlebotomy

SCENARIO An assistant has completed a venous collection for a patient who remains in the collection chair. The patient's printed tube labels are ready, and the clinic requires identity verification and labeling at the collection location.

Which action should occur before the assistant leaves?

A. Match the labels to the patient's verified identifiers and label the collected tubes there.
B. Label only the transport bag because the tubes will stay together.
C. Place unlabeled tubes in a rack for batch labeling after other patients are drawn.
D. Carry the unlabeled tubes to the laboratory and label them from memory.

Q41 | Phlebotomy

SCENARIO A drug-level order specifies collection immediately before the patient's scheduled dose. At the collection visit, the patient says the dose was taken 30 minutes ago. The laboratory instructions say to contact the ordering clinician when timing cannot be met.

What should the assistant do?

A. Document the dose time and contact the ordering clinician for collection instructions.
B. Collect now and label the sample as if it were obtained before the dose.
C. Adjust the reported drug concentration based on the elapsed 30 minutes.
D. Tell the patient to skip tomorrow's dose and return without prescriber approval.

Q42 | Patient Intake and Vitals

> **SCENARIO** A patient brings prescription bottles, an over-the-counter sleep product, and an herbal supplement to a rooming visit. The EHR lists only two prescriptions.

What should the assistant include when updating the medication history?

A. Only prescription medications supplied by the patient's current provider.
B. Only products the assistant considers likely to affect today's complaint.
C. Each reported product, its use details, and discrepancies for clinician review.
D. The prior list unchanged until the patient has a new prescription.

Q43 | General Patient Care

> **SCENARIO** An assistant sets up for a minor procedure in which a specimen will be collected. The verified order specifies a sterile container without fixative, but the tray contains a formalin-filled jar.

Which preparation is appropriate?

A. Pour out the formalin and use the same jar without further review.
B. Replace the formalin jar with the ordered sterile container without fixative.
C. Cancel the specimen order and use a dry paper envelope.
D. Use the formalin jar because it preserves every type of specimen.

Q44 | General Patient Care

> **SCENARIO** A patient with a painful shoulder cannot tolerate the standard position for an ordered minor skin procedure. The provider has not yet entered the room.

What should the assistant do?

A. Administer an unrequested sedative so the patient can tolerate positioning.
B. Cancel the procedure and tell the patient no alternative is possible.
C. Explain the limitation to the provider and obtain an acceptable positioning adjustment.
D. Force the standard position briefly because the procedure will be short.

Q45 | Patient Intake and Vitals

> **SCENARIO** An adult reports dizziness. A manual blood pressure measurement is 82/48 mm Hg; clinic procedure requires immediate clinician notification for systolic pressure below 90 mm Hg with symptoms.

What should the assistant do?

A. Record 120/80 mm Hg because the low result is probably equipment error.
B. Delay reporting until the provider reviews all roomed patients.
C. Keep the patient safely seated or lying down and immediately report the value and dizziness.
D. Ask the patient to walk around before deciding whether to report the value.

Q46 | Phlebotomy

> **SCENARIO** An adult needs routine venipuncture. The assistant sees several blue lines through the skin, but palpation identifies a straight, resilient median cubital vein away from areas of injury.

Which finding should guide site selection?

A. The palpated course and condition of the suitable median cubital vein.
B. A readily visible vessel within a recently bruised area.
C. A straight but hardened vessel that feels fixed under the skin.
D. The darkest visible skin line regardless of what can be palpated.

Q47 | Phlebotomy

> **SCENARIO** An assistant will collect capillary blood from several clinic patients using single-use, auto-disabling safety lancets. One lancet has already been activated for the first patient.

Which supply choice is appropriate for the next patient?

A. The used lancet after following the meter's surface-disinfection procedure.
B. A second patient's reusable lancing device with a fresh lancet tip.
C. An unopened, new single-use safety lancet.
D. The used lancet after wiping its tip with alcohol.

Q48 | General Patient Care

> **SCENARIO** A provider's signed oral medication order lists a drug and frequency but omits the dose. The patient says the dose is probably the same as last year.

What should the assistant do before administration?

A. Obtain a complete, clarified order from the authorized prescriber.
B. Copy the dose from a different patient's order for the same drug.
C. Select the lowest dose listed on the package.
D. Use the patient's recollection as the missing dose instruction.

Q49 | General Patient Care

> **SCENARIO** The assistant enters a provider's signed order for a left-knee radiograph. Before routing it, the EHR preview displays "right knee."

Which action is appropriate?

A. Change the signed provider order to right knee to match the preview.
B. Route it and tell the patient to ask radiology to change it.
C. Correct the laterality to match the signed order before routing it.
D. Order both knees to avoid deciding which side is correct.

Q50 | Patient Intake and Vitals

> **SCENARIO** The assistant measures an adult's height with a stadiometer. The patient wears thick-soled shoes and looks down at the floor while standing against the device.

Which correction improves the measurement?

A. Remove the shoes and position the head level before lowering the headpiece.
B. Ask the patient to tilt the head farther down before lowering the headpiece.
C. Measure while the patient stands on tiptoe to straighten the spine.
D. Keep the shoes on and subtract an estimated amount afterward.

Q51 | EKG and Cardiovascular Testing

Where is V1 placed for a standard 12-lead EKG?

A. Left midaxillary line at the horizontal level of V4.
B. Fourth intercostal space at the right sternal border.
C. Fourth intercostal space at the left sternal border.
D. Fifth intercostal space at the left midclavicular line.

Q52 | Phlebotomy

> **SCENARIO** A centrifuge manufacturer requires equal-mass loads in directly opposite positions. Only one specimen tube is ready, and the clinic has approved balance tubes that can be matched by mass.

How should the assistant load the centrifuge?

A. Place an empty tube beside the specimen without checking mass.
B. Hold the centrifuge housing firmly while an unbalanced run starts.
C. Run the specimen tube alone at a lower speed.
D. Place an equal-mass approved balance tube directly opposite the specimen tube.

Q53 | Infection Control and Safety

> **SCENARIO** An assistant removes gloves after handling a used dressing and is about to prepare clean supplies for another patient. There is no visible soil on the hands.

Which action is appropriate before handling the clean supplies?

A. Handle the clean supplies first and perform hand hygiene afterward.
B. Perform hand hygiene after glove removal and before the clean task.
C. Put on new gloves without hand hygiene because the old gloves were intact.
D. Wipe the hands on the gown and proceed to the supply drawer.

Q54 | Patient Intake and Vitals

> **SCENARIO** A 79-year-old patient becomes pale and says, "I am going to fall," while standing for ordered orthostatic vital signs. Another trained staff member is nearby.

What should the assistant do next?

A. Ask the patient to walk a few steps before repeating the standing pressure.
B. Help the patient safely sit or lie down and summon the clinician.
C. Leave the patient standing while retrieving a smaller cuff.
D. Keep the patient standing until all timed measurements are complete.

Q55 | General Patient Care

> **SCENARIO** A child is scheduled for a simple dressing change. The provider has approved the care, and the caregiver is present. The child becomes fearful after seeing the supplies.

Which preparation best supports the child's participation?

A. Explain the steps in concrete, age-appropriate terms and offer a limited safe choice.
B. Tell the child the dressing is a punishment for touching the wound.
C. Promise that the procedure cannot cause any discomfort.
D. Ask the caregiver to leave immediately without considering the child's needs.

Q56 | General Patient Care

> **SCENARIO** While preparing to assist with an ordered sterile procedure, the assistant sees a sterile-gloved hand touch the unprepared examination-table rail. The procedure has not started.

Which action restores safe preparation?

A. Keep using the glove because only a small area touched the rail.
B. Wipe the glove on the sterile drape and continue.
C. Treat the glove as contaminated and replace it using the appropriate sterile technique.
D. Spray the glove with surface disinfectant and begin the procedure.

Q57 | Infection Control and Safety

> **SCENARIO** A disinfectant label specifies a 3-minute wet contact time on the examination table. After cleaning visible soil, the assistant applies the product, but the surface dries after 1 minute.

What should the assistant do?

A. Seat the next patient because the surface already looks dry.
B. Dilute the product with extra water to make it last longer.
C. Reapply as directed so the surface remains wet for the full labeled contact time.
D. Wipe the product off immediately with a dry towel.

Q58 | EKG and Cardiovascular Testing

> **SCENARIO** A comfortable patient is shivering in a cold room during a resting EKG. The tracing contains irregular fine spikes superimposed on the complexes; the electrodes and cables are secure.

Which action best addresses the likely artifact?

A. Ask the patient to hold both arms rigidly above the table.
B. Move V1 and V2 down two intercostal spaces.
C. Warm the patient, support relaxed limbs, and repeat the recording when shivering stops.
D. Increase the paper speed without addressing the shivering.

Q59 | General Patient Care

> **SCENARIO** A trained assistant is authorized to give an ordered oral tablet. At the bedside, the package has the prescribed drug name but states extended release; the verified order specifies immediate release.

Which action is appropriate?

A. Administer the tablet because the drug name matches.
B. Hold administration and resolve the formulation mismatch with the clinician.
C. Crush the extended-release tablet to make it act like immediate release.
D. Give half the extended-release tablet to compensate for the formulation.

Q60 | General Patient Care

> **SCENARIO** An assistant trained and authorized to remove sutures begins an ordered removal after provider review. As one suture is removed, the incision edges begin to separate.

What should the assistant do?

A. Remove the remaining sutures rapidly to avoid uneven tension.
B. Stop removal, support the area, and notify the provider immediately.
C. Insert replacement sutures using the instruments on the tray.
D. Tell the patient the separation will close without evaluation.

Q61 | General Patient Care

> **SCENARIO** An assistant trained and authorized to administer oral medication sees an order for 500 mg and a bottle labeled 250 mg per tablet. The tablets are the verified medication, and the patient can swallow tablets.

How many tablets provide the ordered dose?

A. 1 tablet
B. 4 tablets
C. 2 tablets
D. 0.5 tablet

Q62 | General Patient Care

SCENARIO An authorized assistant is preparing a patient's ordered intramuscular injection. The patient points to the intended arm and reports that the skin there is hot, red, and painful.

What is the best next step?

A. Inject into the reddest part because blood flow is increased.
B. Change the ordered intramuscular route to subcutaneous without consultation.
C. Pause and ask the clinician to evaluate the area and confirm a suitable site.
D. Apply an ice pack and inject into the same area once it feels numb.

Q63 | General Patient Care

SCENARIO A patient is leaving with crutches after the provider has selected and ordered them. During the fitting check, the patient rests body weight on the axillary pads and reports hand tingling.

Which response is appropriate?

A. Stop the trial and have the trained clinician reassess fit and weight-bearing technique.
B. Shorten both crutches without checking the ordered fitting procedure.
C. Remove the handgrips so the patient must use the axillary pads.
D. Tell the patient to bear more weight through the axillary pads.

Q64 | Patient Intake and Vitals

SCENARIO A patient weighs 154 lb. The intake form requires kilograms, and its conversion instruction is kg = lb / 2.2, rounded to the nearest whole kilogram.

Which weight should the assistant enter?

A. 70 kg
B. 68 kg
C. 339 kg
D. 75 kg

Q65 | Phlebotomy

SCENARIO A young infant has an order for capillary collection. The clinic's approved neonatal collection diagram specifies the medial or lateral plantar heel surface and prohibits the central and posterior heel. An age-appropriate depth-limited lancet is available.

Which puncture site follows the supplied diagram?

A. The curved posterior heel surface.
B. An already bruised puncture site in the central heel.
C. The lateral plantar surface within the approved heel area.
D. The center of the plantar heel beneath the calcaneus.

Q66 | General Patient Care

SCENARIO An authorized, trained assistant has an order to give acetaminophen 480 mg orally. The verified liquid is labeled 160 mg per 5 mL. No rounding is needed.

What volume is required for the ordered dose?

A. 15 mL
B. 30 mL
C. 5 mL
D. 10 mL

Q67 | General Patient Care

SCENARIO A patient waiting for a procedure has a seizure with generalized jerking. The assistant activates the emergency response, and staff begin clearing nearby equipment.

Which additional action protects the patient?

A. Give water during the jerking to prevent dehydration.
B. Protect the head from injury and observe the duration without restraining the movements.
C. Hold the arms and legs tightly against the examination table.
D. Place a tongue depressor between the teeth to keep the mouth open.

Q68 | Point of Care Testing and Laboratory Procedures

SCENARIO A patient is scheduled for distance visual-acuity screening. The clinic protocol says to test with the patient's usual distance correction and record whether correction was used. The patient normally wears distance glasses.

How should the assistant perform and document the screening?

A. Remove the glasses and document the test as corrected vision.
B. Use the prior corrected acuity result in place of today's screening.
C. Test through the distance glasses but document the result as uncorrected.
D. Test with the distance glasses and document that correction was worn.

Q69 | Point of Care Testing and Laboratory Procedures

SCENARIO A provider orders a clean-catch midstream urine specimen. The patient asks whether to begin urinating directly into the collection cup. The supplied instructions call for initial urine into the toilet, then midstream collection without touching the inside of the cup.

Which response reinforces the correct technique?

A. Collect the first drops directly into the cup before any urine enters the toilet.
B. Begin in the toilet, then collect the midstream portion without touching the cup interior.
C. Touch the cup interior to the skin so no urine spills.
D. Fill the cup from the toilet bowl after voiding.

Q70 | EKG and Cardiovascular Testing

SCENARIO For a standard 12-lead EKG, V4 is correctly placed in the fifth intercostal space at the left midclavicular line. The assistant now needs to place V6.

Which position is correct for V6?

A. Left anterior axillary line at the same horizontal level as V4.
B. Right midaxillary line at the same horizontal level as V4.
C. Left midaxillary line at whichever intercostal space lies directly below V4.
D. Left midaxillary line at the same horizontal level as V4.

Q71 | Infection Control and Safety

SCENARIO A used injection needle has a built-in safety shield. The assistant has completed the injection and is beside an approved sharps container.

Which disposal action is appropriate?

A. Place the exposed needle on the tray for later disposal.
B. Recap the needle with both hands before carrying it to the container.
C. Remove the needle by hand and place it in a regular wastebasket.
D. Activate the safety feature as instructed and discard the device promptly into the sharps container.

Q72 | Point of Care Testing and Laboratory Procedures

SCENARIO A patient performing ordered spirometry stops blowing after 1 second, although the device indicates that expiration is incomplete. The written protocol requires an acceptable sustained effort and repeat coaching before a result is accepted.

What should the assistant do?

A. Explain and demonstrate the sustained exhalation again, then repeat according to protocol.
B. Select the largest displayed number from the failed attempts as the final result.
C. Accept the brief effort as the final result because some air was exhaled.
D. Record the incomplete effort as acceptable if its first second looks smooth.

Q73 | Point of Care Testing and Laboratory Procedures

SCENARIO Before ordered skin allergy testing, the clinic protocol requires clinician review if an antihistamine was taken within the past 48 hours. The patient reports taking an antihistamine that morning and has no current symptoms.

What should the assistant do before testing begins?

A. Proceed with testing and omit the medication because the patient feels well.
B. Increase the test allergen amount to compensate for the antihistamine.
C. Tell the patient to stop every prescription for a week and return without clinician review.
D. Document the medication and time taken and obtain the testing clinician's instructions.

Q74 | Patient Intake and Vitals

> **SCENARIO** A patient rates knee pain as 8 on a 0-10 scale, sits quietly, and smiles during conversation. The assistant sees no visible injury.

How should the assistant document the pain?

A. Lower the rating to 3 because the patient is smiling.
B. Record the patient's rating and relevant reported features of the pain.
C. Record severe tissue damage as the cause of the reported pain.
D. Omit the rating until a visible cause is identified.

Q75 | Infection Control and Safety

> **SCENARIO** A blood glucose meter is approved by its manufacturer for use on multiple patients. Its instructions require cleaning and disinfection after every patient using the specified compatible product.

What should the assistant do before using it for the next patient?

A. Place the meter in a clean bag without disinfecting it.
B. Change the test strip without processing the meter.
C. Complete the specified cleaning and disinfection process.
D. Wipe only the display with a dry tissue.

Q76 | EKG and Cardiovascular Testing

> **SCENARIO** A regular EKG is recorded at 25 mm/s. The supplied reference states that each large box represents 0.2 seconds at this speed. Consecutive R waves are five large boxes apart.

What ventricular rate does this interval represent?

A. 75 beats/min
B. 100 beats/min
C. 150 beats/min
D. 60 beats/min

Q77 | General Patient Care

Which instrument is designed to remove skin staples?

A. A skin staple remover.
B. A hemostat.
C. A needle holder.
D. Fine tissue forceps.

Q78 | Infection Control and Safety

> **SCENARIO** A sealed sterile pack is found wet after storage beneath a leaking pipe. The wrapper is otherwise intact, and the pack is needed for a procedure.

Which action is appropriate?

A. Place a dry towel around the pack and use it immediately.
B. Let the wrapper dry and return the pack to sterile stock.
C. Use only instruments from the center of the wet pack.
D. Remove the pack from sterile stock and obtain an uncompromised pack.

Q79 | Patient Intake and Vitals

> **SCENARIO** A comfortable adult is seated for a routine blood pressure measurement with crossed legs, an unsupported back, and the cuffed arm hanging beside the chair.

Which setup best supports an accurate resting measurement?

A. Keep the legs crossed and support the arm at heart level after the patient rests.
B. Support the back and arm at heart level, with feet flat and legs uncrossed.
C. Support the back and feet but leave the cuffed arm hanging freely at the side.
D. Have the patient lean forward and hold the cuffed arm unsupported at chest height.

Q80 | EKG and Cardiovascular Testing

> **SCENARIO** During EKG preparation, a patient becomes dizzy and reports chest discomfort. The monitor displays a rate of 38 beats/min with a stable signal, and the assistant can feel a slow pulse.

What should the assistant do next?

A. Finish cleaning every cable before informing the clinician.
B. Summon the clinician immediately and activate the clinic emergency response as indicated.
C. Choose an antiarrhythmic medication based on the displayed rate.
D. Assume electrical interference and leave the patient alone to obtain another machine.

Q81 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A test's collection instructions require a specific transport swab and medium. The assistant finds that the swab just collected from the throat is in a different, incompatible transport medium; no result has been produced.

Which action is appropriate?

A. Transfer the used swab into the correct medium and assume the sample is acceptable.
B. Notify the clinician and arrange recollection with the required collection system.
C. Send the incompatible system without documenting the medium problem.
D. Hold the specimen until courier pickup and substitute the correct medium then.

Q82 | Infection Control and Safety

SCENARIO A patient arrives coughing, reports recent fever, and is waiting near other patients. The clinic respiratory-hygiene procedure directs staff to offer a mask if tolerated, separate symptomatic patients, and promptly notify clinical staff.

Which action follows the procedure?

A. Delay notifying clinical staff until the patient has completed all paperwork.
B. Ask the patient to remove a tolerated mask when speaking to registration.
C. Keep the patient in the crowded waiting area until the cough stops.
D. Offer a mask, move the patient to the designated separate area, and alert clinical staff.

Q83 | Phlebotomy

SCENARIO After venipuncture, a patient taking an anticoagulant still has oozing at the puncture site. The assistant is holding clean gauze over the site, and the patient is otherwise stable.

Which action is appropriate?

A. Ask the patient to bend the elbow tightly and leave without checking the site.
B. Apply a bandage over the actively oozing site and send the patient away.
C. Tell the patient to stop the anticoagulant for the next week.
D. Maintain direct pressure, monitor for hemostasis, and notify the clinician if bleeding persists.

Q84 | General Patient Care

SCENARIO An authorized, trained assistant is preparing an ordered 0.75 mg intramuscular dose. The verified vial contains 1.5 mg/mL, and a syringe graduated in 0.1 mL increments is available. No rounding is needed.

What volume corresponds to the order?

A. 0.05 mL
B. 1 mL
C. 0.5 mL
D. 2 mL

ANSWER KEY & EXPLANATIONS

Practice Exam 03 | Domain 3: Clinical Patient Care | 84 scored questions

01 A	02 C	03 A	04 A	05 B	06 A
07 A	08 D	09 D	10 D	11 A	12 B
13 A	14 C	15 B	16 A	17 A	18 C
19 D	20 B	21 A	22 D	23 A	24 B
25 D	26 D	27 C	28 B	29 D	30 A
31 D	32 B	33 C	34 C	35 A	36 D
37 D	38 B	39 B	40 A	41 A	42 C
43 B	44 C	45 C	46 A	47 C	48 A
49 C	50 A	51 B	52 D	53 B	54 B
55 A	56 C	57 C	58 C	59 B	60 B
61 C	62 C	63 A	64 A	65 C	66 A
67 B	68 D	69 B	70 D	71 D	72 A
73 D	74 B	75 C	76 D	77 A	78 D
79 B	80 B	81 B	82 D	83 D	84 C

DETAILED EXPLANATIONS

Q01 | Correct answer: A

The number of beats counted over 60 seconds and the observed irregularity.

A full-minute count improves measurement when beat spacing is irregular and follows the stated procedure. Short counts can misrepresent the rate, and the irregularity should be reported rather than hidden.

Q02 | Correct answer: C

Review the stated warning sign again and ask the patient to explain when to call.

The patient's response shows misunderstanding of an explicit provider instruction. Reinforcement and another teach-back check support understanding without creating a new treatment plan.

Q03 | Correct answer: A

Sterile normal saline and a nonadherent dressing pad.

Sterile normal saline and a nonadherent pad match both specified parts of the ordered procedure. A different cleansing solution or dressing type does not fulfill the supplied instructions.

Q04 | Correct answer: A

Stop routine intake, stay with the patient, and activate the clinic emergency response.

New chest pressure with breathing difficulty and sweating requires immediate clinical attention. Completing routine rooming tasks or leaving the patient alone delays evaluation.

Q05 | Correct answer: B

Stop patient testing, document the failed control, and troubleshoot under the instructions.

The failed control means the stated release condition has not been met. Waived status does not eliminate the need to follow instructions, and adjusting patient values by the control difference is not a validated correction.

Q06 | Correct answer: A

Follow the safety data sheet and trained spill procedure using the specified kit and PPE.

The product-specific safety data sheet determines appropriate protection and spill handling. Mixing chemicals or improvising disposal can create additional hazards.

Q07 | Correct answer: A

Wash the puncture with soap and water, report the exposure, and obtain urgent occupational evaluation.

A contaminated puncture needs immediate local care and prompt reporting and medical evaluation. Waiting for symptoms or independently arranged source testing can delay time-sensitive exposure management.

Q08 | Correct answer: D

Confirm the current hotel location and callback number along with identity and connection.

The stated procedure requires the patient's actual location for the session, which may differ from the home address. A portal connection does not remove the need for the specified identity and contact checks.

Q09 | Correct answer: D

Pause and report the new history to the ordering clinician for review.

The new history may change whether irrigation is appropriate and requires clinician review. Adjusting pressure or temperature does not address the unreviewed risk.

Q10 | Correct answer: D

Obtain the second trained helper and prepare the wheelchair and transfer area.

The documented transfer plan calls for two trained helpers and an appropriately prepared environment. Pulling on the arms or using footrests as steps increases injury and fall risk.

Q11 | Correct answer: A

Resolve the site discrepancy with the provider and obtain a correct label before transfer.

A specimen label must match the identified source site as well as the patient. Resolving the discrepancy before transfer prevents a misleading specimen record.

Q12 | Correct answer: B

Pause the screening and restore an environment that meets the device instructions.

Background noise can mask screening tones and invalidate responses. Arbitrarily changing and misreporting settings or coaching responses does not correct the testing environment.

Q13 | Correct answer: A

The needle holder.

A needle holder is designed to grasp the needle securely during suturing. Tissue forceps hold tissue and do not replace the designated needle-control instrument.

Q14 | Correct answer: C

Apply continuous direct pressure using clean gauze and report persistent bleeding.

Continuous direct pressure helps control external bleeding. Repeatedly lifting the gauze interrupts pressure and can disturb early clot formation.

Q15 | Correct answer: B

Remove the damaged lamp from service under policy and use the intact replacement.

Exposed wiring requires removal from service and the approved repair process. A bandage or careful positioning does not make damaged electrical equipment safe.

Q16 | Correct answer: A

Observe chest movement discreetly while appearing to continue the pulse assessment.

Discreet observation helps measure the patient's spontaneous breathing pattern. Instructed deep breathing or conversation changes the pattern being measured.

Q17 | Correct answer: A

Place the labeled tube in the approved light-protective sleeve and transport at room temperature.

The supplied instruction requires both light protection and room-temperature transport. Freezing is not an equivalent substitute and may violate the specimen's handling requirements.

Q18 | Correct answer: C

Flush the eye with water, then promptly report and obtain exposure evaluation.

Exposed mucous membranes should be flushed with water promptly, followed by reporting and evaluation. Alcohol wipes and surface disinfectants can damage the eye and are not appropriate first aid.

Q19 | Correct answer: D

Record the request and symptoms and route them to the prescriber for a decision.

A requested dose increase requires prescriber review. The assistant can gather and route information but cannot independently change the medication plan.

Q20 | Correct answer: B

Correct the item selection to match the signed walker order.

The equipment request must match the authorized clinical order. A cane and a walker provide different support and cannot be substituted on the assistant's initiative.

Q21 | Correct answer: A

In the designated regulated-waste container.

The supplied plan classifies compressibly blood-saturated material as regulated waste. Gauze is not a sharp merely because it contains blood, and clean recycling is inappropriate.

Q22 | Correct answer: D

Treat it as invalid and repeat or escalate according to the test instructions.

The missing required control line makes the test invalid rather than positive or negative. Reading after the specified window does not rescue a failed validity check.

Q23 | Correct answer: A

Clean the instrument according to its instructions before sterilization.

Cleaning removes material that can interfere with subsequent sterilization. A longer cycle cannot be substituted for the required removal of visible soil.

Q24 | Correct answer: B

Record the reported antibiotic and reaction, and alert the clinician before medication tasks.

The reported drug and specific reaction are clinically important and should be brought to the clinician's attention. Relabeling the event as a nonspecific intolerance loses information, and an assistant cannot independently perform a drug challenge.

Q25 | Correct answer: D

Gloves, a protective gown, and eye and nose-mouth protection.

Standard Precautions call for PPE matched to the expected exposure. Gloves alone do not protect the eyes, nose, mouth, or clothing from an anticipated splash.

Q26 | Correct answer: D

Explain the leg supports and draping, then help the patient assume the ordered position safely.

Lithotomy positioning uses supported legs and appropriate draping for the examination. Clear instructions and safe assistance protect both patient comfort and access to the examination area.

Q27 | Correct answer: C

Warm the hand, reposition the sensor as directed, and obtain a stable signal.

Cold-related poor perfusion can impair pulse oximeter readings. A stable signal after correcting technique is more useful than selecting or averaging unreliable numbers; any concerning symptoms still require immediate reporting.

Q28 | Correct answer: B

Arrange recollection using a tube filled to the required mark.

The supplied fill requirement is necessary for the specified blood-to-additive ratio. Adding another tube's blood or water changes the specimen and cannot correct the collection.

Q29 | Correct answer: D

Sodium citrate, then serum, then EDTA.

The specified sequence places citrate before serum and EDTA last. Following the laboratory's additive-based instructions helps avoid errors from relying on tube colors that may vary by product.

Q30 | Correct answer: A

Stop the draw, release the tourniquet, remove the needle safely, and apply pressure.

Pain and swelling during collection suggest blood is entering surrounding tissue and require stopping the draw. Continuing or manipulating the needle can worsen injury.

Q31 | Correct answer: D

Left deltoid muscle.

The anatomical site identifies where the injection was administered. Tolerance and follow-up may be useful elsewhere, but they do not identify the injection site.

Q32 | Correct answer: B

Activate the clinic emergency response and summon the provider while staying with the patient.

Rapid swelling and respiratory symptoms after an injection require immediate emergency escalation. Oral intake and routine discharge tasks delay care and may be unsafe when speaking or breathing is difficult.

Q33 | Correct answer: C

35-44 cm

The measured arm circumference falls within the 35-44 cm cuff range. A cuff that does not fit its labeled circumference range can produce an inaccurate result.

Q34 | Correct answer: C

Discard the remainder according to policy and obtain a new supply for the second patient.

Single-dose medications are not to be shared between patients. A fresh needle does not convert the remaining contents into a multidose product.

Q35 | Correct answer: A

Sterilization using the validated process.

An instrument entering sterile tissue requires sterilization, using a method compatible with the device. Cleaning is necessary preparation but does not by itself provide the required processing level.

Q36 | Correct answer: D

Set the paper speed to 25 mm/s and verify the printed settings before routing the tracing.

The acquisition settings should match the requested speed and gain, with accurate printed calibration information. Changing gain does not correct a paper-speed mismatch.

Q37 | Correct answer: D

Ask the patient to state full name and date of birth and match both to the chart.

Having the patient state two identifiers helps distinguish people with the same name before data enter a chart. Appointment times, room assignments, and a yes response to a suggested name do not reliably establish identity.

Q38 | Correct answer: B

Use the reclining setup and follow the clinic's fainting-risk procedure before collection.

Planning a supported reclining position addresses the known risk before symptoms occur. Waiting until loss of consciousness or using an unsupported seat increases the chance of a fall.

Q39 | Correct answer: B

Notify the designated clinician immediately, confirm read-back, and document the communication.

The stated critical-result process requires active communication, confirmation, and documentation. Passive chart entry does not complete that process, and the assistant must not create a treatment change.

Q40 | Correct answer: A

Match the labels to the patient's verified identifiers and label the collected tubes there.

Labeling verified specimens at the collection location reduces mix-ups and preserves identification. Labeling only a bag leaves the individual tubes unidentified when removed.

Q41 | Correct answer: A

Document the dose time and contact the ordering clinician for collection instructions.

The actual dose time no longer matches the ordered collection timing, so the stated escalation process applies. The assistant should not relabel timing, adjust results, or independently alter medication dosing.

Q42 | Correct answer: C

Each reported product, its use details, and discrepancies for clinician review.

Medication reconciliation includes nonprescription products and supplements because they may affect care. The assistant records reported use and flags discrepancies without independently deciding which therapies to stop.

Q43 | Correct answer: B

Replace the formalin jar with the ordered sterile container without fixative.

Specimen preparation must match the test order, because fixatives can make some testing impossible. Emptying a fixative jar does not make it equivalent to the requested sterile container.

Q44 | Correct answer: C

Explain the limitation to the provider and obtain an acceptable positioning adjustment.

The provider can determine a safe position compatible with the procedure and the patient's limitation. The assistant should not force painful movement or independently prescribe medication or cancel treatment.

Q45 | Correct answer: C

Keep the patient safely seated or lying down and immediately report the value and dizziness.

The measured pressure and symptoms meet the supplied escalation rule. The assistant should protect the patient and report promptly without diagnosing the cause or substituting a normal value.

Q46 | Correct answer: A

The palpated course and condition of the suitable median cubital vein.

Palpation helps establish the course and suitability of a vein rather than relying on skin color or visibility alone. Hardened or injured areas are poor choices for routine collection.

Q47 | Correct answer: C

An unopened, new single-use safety lancet.

A new single-use, auto-disabling lancet is appropriate for assisted collection. Neither disinfecting a used lancet nor changing the tip of another patient's reusable lancing device makes sharing acceptable.

Q48 | Correct answer: A

Obtain a complete, clarified order from the authorized prescriber.

A missing dose prevents safe verification of the medication task. The assistant must obtain prescriber clarification rather than infer a dose from memory or another patient's treatment.

Q49 | Correct answer: C

Correct the laterality to match the signed order before routing it.

Laterality is a safety-critical part of the ordered procedure and should match before transmission. Ordering extra views or altering the provider's intended side is not an appropriate workaround.

Q50 | Correct answer: A

Remove the shoes and position the head level before lowering the headpiece.

Shoes and head position can change measured standing height. Direct measurement with appropriate positioning is more reliable than an estimated correction.

Q51 | Correct answer: B

Fourth intercostal space at the right sternal border.

V1 belongs in the fourth intercostal space at the right sternal border. The corresponding left sternal position is V2, not V1.

Q52 | Correct answer: D

Place an equal-mass approved balance tube directly opposite the specimen tube.

An equal-mass opposite load follows the supplied balancing requirement. A lower speed or an unverified empty tube does not establish a balanced rotor.

Q53 | Correct answer: B

Perform hand hygiene after glove removal and before the clean task.

Gloves do not replace hand hygiene, including after removal and before a clean task. A new pair of gloves does not correct contamination acquired during glove use or removal.

Q54 | Correct answer: B

Help the patient safely sit or lie down and summon the clinician.

Preventing a fall takes precedence over completing orthostatic measurements. The assistant should obtain help and report the symptoms rather than prolonging the unsafe position.

Q55 | Correct answer: A

Explain the steps in concrete, age-appropriate terms and offer a limited safe choice.

Concrete explanations and a limited choice, such as which arm rests on a pillow, help a child understand and participate. Promising no discomfort can undermine trust if the procedure is uncomfortable.

Q56 | Correct answer: C

Treat the glove as contaminated and replace it using the appropriate sterile technique.

Contact with an unprepared surface contaminates the sterile glove. Wiping or applying surface disinfectant does not restore the glove's sterility.

Q57 | Correct answer: C

Reapply as directed so the surface remains wet for the full labeled contact time.

The disinfectant must remain wet for its labeled contact time to be used effectively. Appearance and drying alone do not show that the specified disinfection process was completed.

Q58 | Correct answer: C

Warm the patient, support relaxed limbs, and repeat the recording when shivering stops.

Shivering produces muscle activity that can contaminate the tracing. Warming and relaxation target that source, whereas changing anatomical electrode positions creates a new recording error.

Q59 | Correct answer: B

Hold administration and resolve the formulation mismatch with the clinician.

The formulation is part of medication verification and cannot be assumed interchangeable. Crushing or changing the amount can alter drug delivery and does not resolve the order mismatch.

Q60 | Correct answer: B

Stop removal, support the area, and notify the provider immediately.

New separation requires prompt provider reassessment and protection of the wound. Continuing removal can worsen separation, and placing new sutures exceeds this authorized removal task.

Q61 | Correct answer: C

2 tablets

500 mg / 250 mg per tablet = 2 tablets. Giving one tablet would provide half the ordered dose.

Q62 | Correct answer: C

Pause and ask the clinician to evaluate the area and confirm a suitable site.

A painful, inflamed area requires review before injection. The assistant should obtain a suitable site decision within the order and cannot independently change the route.

Q63 | Correct answer: A

Stop the trial and have the trained clinician reassess fit and weight-bearing technique.

Weight through the axillary pads can place pressure on nerves and vessels. Tingling warrants stopping the trial and obtaining a proper fitting and technique review rather than encouraging more axillary loading.

Q64 | Correct answer: A

70 kg

154 lb / 2.2 lb per kg = 70 kg. Multiplying by 2.2 converts in the opposite direction and does not provide the requested kilogram value.

Q65 | Correct answer: C

The lateral plantar surface within the approved heel area.

The lateral plantar area is one of the explicitly approved sites in the supplied neonatal procedure. The central and posterior heel are excluded, and an injured site should not be chosen.

Q66 | Correct answer: A

15 mL

480 mg x 5 mL / 160 mg = 15 mL. A 5 mL volume provides only 160 mg, while 30 mL provides twice the ordered dose.

Q67 | Correct answer: B

Protect the head from injury and observe the duration without restraining the movements.

Protecting the patient from nearby hazards and noting seizure duration support emergency care. Restraint, objects in the mouth, and oral fluids can cause injury.

Q68 | Correct answer: D

Test with the distance glasses and document that correction was worn.

Using and documenting the usual distance correction follows the supplied screening procedure. Corrected and uncorrected findings are not interchangeable and should not be mislabeled.

Q69 | Correct answer: B

Begin in the toilet, then collect the midstream portion without touching the cup interior.

The supplied clean-catch procedure uses a midstream portion while protecting the container interior from contamination. Collecting the initial drops or toilet-bowl urine does not follow that procedure.

Q70 | Correct answer: D

Left midaxillary line at the same horizontal level as V4.

V6 is positioned on the left midaxillary line, horizontally aligned with V4. The left anterior axillary location at that level corresponds to V5.

Q71 | Correct answer: D

Activate the safety feature as instructed and discard the device promptly into the sharps container.

Prompt disposal with the device safety feature reduces exposure to a contaminated sharp. Routine two-handed recapping, manual removal, or leaving the sharp exposed creates avoidable injury risk.

Q72 | Correct answer: A

Explain and demonstrate the sustained exhalation again, then repeat according to protocol.

A prematurely ended effort does not meet the supplied acceptance criterion. Coaching and an appropriate repeat are preferable to estimating missing data or accepting an unacceptable maneuver.

Q73 | Correct answer: D

Document the medication and time taken and obtain the testing clinician's instructions.

The reported medication use triggers the explicitly supplied pretest review rule. The assistant should obtain clinician instructions rather than independently changing medications or modifying the test.

Q74 | Correct answer: B

Record the patient's rating and relevant reported features of the pain.

Pain ratings reflect the patient's report and should be documented with location and other relevant details. Appearance alone does not justify changing the score or assigning a diagnosis.

Q75 | Correct answer: C

Complete the specified cleaning and disinfection process.

A shared meter can carry contamination even when a fresh strip is used. The manufacturer's complete between-patient processing instructions must be followed.

Q76 | Correct answer: D

60 beats/min

Five large boxes x 0.2 seconds per box = 1 second per beat. 60 seconds per minute / 1 second per beat = 60 beats/min; this calculation describes rate without selecting a diagnosis or treatment.

Q77 | Correct answer: A

A skin staple remover.

A staple remover lifts and reshapes skin staples for removal. A needle holder is used to control a suturing needle and is not the designated staple-removal instrument.

Q78 | Correct answer: D

Remove the pack from sterile stock and obtain an uncompromised pack.

Moisture can compromise a sterile barrier and allow contamination. Drying the outside does not establish that the contents remained sterile.

Q79 | Correct answer: B

Support the back and arm at heart level, with feet flat and legs uncrossed.

Back support, uncrossed legs, flat feet, and an arm supported at heart level reduce positioning errors. Holding an arm up without support is not equivalent to resting it on a support.

Q80 | Correct answer: B

Summon the clinician immediately and activate the clinic emergency response as indicated.

Symptoms with a confirmed slow pulse require prompt patient-focused escalation. Equipment tasks must not delay care, and selecting cardiac medication is beyond the assistant's role.

Q81 | Correct answer: B

Notify the clinician and arrange recollection with the required collection system.

The specified collection system is part of the test's validated handling requirements. Relabeling or transferring after incompatible exposure does not establish acceptability.

Q82 | Correct answer: D

Offer a mask, move the patient to the designated separate area, and alert clinical staff.

The stated procedure combines source control, separation, and prompt clinical notification. Waiting for paperwork leaves other patients exposed and delays evaluation.

Q83 | Correct answer: D

Maintain direct pressure, monitor for hemostasis, and notify the clinician if bleeding persists.

Continued direct pressure and confirmation that bleeding has stopped are important before departure. The assistant should report persistent bleeding and must not independently stop prescribed anticoagulant therapy.

Q84 | Correct answer: C

0.5 mL

0.75 mg / 1.5 mg per mL = 0.5 mL. The order and supplied concentration must be used with their units; a decimal-place error would give a tenfold underdose.

End of Practice Exam